



Commentary

Mind the (widening) gap: why public health must engage with AI now

Paula del Rey Puech^{a,*}, Rebecca Payne^b, Jasjot Saund^c, Martin McKee^d^a Department of Health Services Research and Policy, London School of Hygiene & Tropical Medicine, United Kingdom^b North Wales Centre for Primary Care Research, Bangor University; Nuffield Department of Primary Health Care Sciences, University of Oxford; Betsi Cadwaladr University Health Board, United Kingdom^c Artificial Intelligence Centre for Value-Based Healthcare, London, United Kingdom^d European Observatory on Health Systems and Policies, London School of Hygiene & Tropical Medicine, United Kingdom

ARTICLE INFO

Keywords:

Artificial Intelligence
Machine Learning
Public Health
Equity

ABSTRACT

Objectives: This commentary aims to highlight the opportunities and challenges that Artificial Intelligence (AI) presents for public health.**Study design:** Narrative commentary and conceptual analysis.**Methods:** The commentary draws on material developed for a forthcoming book by the European Observatory on Health Systems and Policies. Sources were selected to highlight both the potential and the limitations of AI integration at population levels, with a focus on equity, governance, and implementation. The analysis is informed by established public health principles: prevention, systems thinking, and the social determinants of health.**Results:** AI applications in public health go beyond process automation and operational efficiency. By integrating and processing diverse, multi-modal data sources, its implementation presents opportunities to understand the wider determinants of health at a more nuanced level and identify populations at risk with greater precision. Additionally, AI has the potential to help understand and support behaviour change in sophisticated ways, enhance disease surveillance and modelling, and enable more targeted and responsive public communication and engagement strategies. However, there are several barriers to realise AI's potential in public health, including system fragmentation, data access limitations, resource constraints, implementation challenges, workforce readiness gaps, and technological limitations such as bias and generative AI "hallucinations".**Conclusion:** Without deliberate engagement, AI risks reinforcing existing inequities. Practical steps for action include embedding AI training in public health education, building multidisciplinary teams, investing in data infrastructure, and ensuring participatory approaches. AI will continue to shape public health systems, whether or not public health professionals engage. We argue that the public health community is both uniquely positioned and ethically obligated to engage proactively with AI.

1. Introduction

Artificial Intelligence (AI) is increasingly being framed as a silver bullet for solving many of our health systems' challenges. With promises ranging from enhanced diagnostic accuracy to the automation of routine clinical tasks, AI is attracting growing attention and investment across the health sector.^{1–3} There is, however, a danger that public health will be left behind. As Buse and colleagues have argued, when the narrative tilts towards precision medicine and individual-level interventions, public health, with its focus on populations, must assert its relevance

and actively shape AI's development in ways that promote health and equity at population and system levels.⁴

This transformation is taking place in the context of a surge in global initiatives and frameworks in recent years aimed at guiding digital transformation in public health. The WHO Global Strategy on Digital Health (2020–2025) provides a roadmap for countries to integrate digital health into health systems, emphasising equity, governance, and capacity-building.⁵ The Global Initiative on Digital Health (GIDH), a network managed by WHO, seeks to align resources and accelerate country-led digital health transformation.⁶ Other international

This article is part of a special issue entitled: AI and Public Health published in Public Health.

* Corresponding author.

E-mail address: paula.delreypuech@nhs.net (P. del Rey Puech).

<https://doi.org/10.1016/j.puhe.2025.106047>

Received 12 August 2025; Received in revised form 17 October 2025; Accepted 23 October 2025

Available online 12 November 2025

0033-3506/© 2025 The Authors. Published by Elsevier Ltd on behalf of The Royal Society for Public Health. This is an open access article under the CC BY license (<http://creativecommons.org/licenses/by/4.0/>).

collaborations, such as the OECD's digital health policy work⁷ and the Global Digital Health Partnership (GDHP),⁸ promote best practices, interoperability, and ethical standards.

This commentary argues that the public health community is both uniquely positioned and ethically obligated to engage with AI. However, doing so requires a deliberate, collective shift: from passive observation (or active disengagement) to proactive engagement.

2. Methods

In writing this commentary we have drawn on our work undertaken for a forthcoming book, to be published by the European Observatory on Health Systems and Policies. In selecting material we have prioritised sources that address both the opportunities and challenges of AI integration at the population level, with particular attention to equity, governance, and real-world implementation. Our conceptual framing is informed by established public health principles, including prevention, systems thinking, and the social determinants of health, to ensure that the discussion remains grounded in the core values and priorities of the field.

3. Results

3.1. Why AI matters for public health

The scope of AI applications in health is expanding rapidly. Until recently, the field has been dominated by tools for purposes such as risk prediction, diagnostic imaging, and operational efficiency.^{3,9} However, advanced computational capacity, data availability, and technological innovations are driving more sophisticated applications. These include agentic AI, which can organise workflows autonomously, and generative AI, which can produce text and images, paving the way for a range of new uses.^{10,11} Other examples, already adopted, include AI-powered mental health chatbots and ambient scribes in primary care.^{12,13} As the pace of investment, development, and adoption of AI systems accelerates, much of this energy is directed towards individualised interventions, such as personalised diagnostics and treatment.¹⁴

For public health professionals, the implications are twofold. First, a shift towards biomedical AI potentially deprioritises investments in prevention and broader determinants of health. Second, the challenges that AI introduces, such as algorithmic bias, inequitable data infrastructures, and opaque decision-making, are not solely technical; they are systemic, and risk worsening health disparities that countries are already struggling to address.¹⁵

Crucially, public health skills and tools are highly relevant to AI development and implementation. Many AI methods, including machine learning, are built on statistical principles familiar to public health, such as probability theory or regression. Rigorous evaluation, a core public health competency, is essential to assess whether AI tools replicate or even accentuate existing inequalities. Such inequalities are at the heart of much public health research and practice, and thus participatory methods are urgently needed to democratise and decentralise AI development. Public health professionals possess the core capabilities required to engage meaningfully with AI, yet too few currently do.

3.2. AI applications with direct relevance to public health

Many recognised benefits of AI stem from its ability to boost work-force productivity by automating tasks. For example, by transcribing and summarising notes from meetings or patient consultations, or increasing the efficiency of diagnostic tools.^{16,17} However, AI's value to public health extends further, offering the opportunity to uncover previously hidden patterns and relationships that can inform more effective interventions.^{18,19} This capability paves the way for more integrated, data-driven strategies to understand and improve population health.

AI techniques can interpret diverse, multi-modal data sources, such

as electronic health records, social care systems, environmental monitoring, social media, and wearable technologies.^{20–22} By integrating clinical data with information on the wider determinants of health, AI could enable us to better forecast disease burden and identify populations at risk, while deepening our understanding of the complex factors influencing health.^{23,24} These tools can also contribute to life-course research and improve our understanding of multi-morbidity.²⁵ Machine learning methods have also been used to help identify neighbourhoods at higher risk of overdose deaths to inform prevention resource allocation.²⁶

In infectious disease epidemiology, AI has the potential to strengthen surveillance systems and improve disease modelling.²⁷ For example, the UK Health Security Agency has evaluated the use of large language models (LLMs) to screen online restaurant reviews for key terms related to gastrointestinal symptoms, to detect foodborne gastrointestinal illness.²⁸ In addition, deep learning techniques have the potential to offer new insights into infection transmission dynamics.²⁹

In health promotion, AI can bring opportunities to support individual behaviour change, for example, by providing new ways to offer smoking cessation or physical activity advice. For example, the WHO has introduced SARA, a digital health promoter that provides advice about healthier choices in multiple languages by leveraging generative AI.^{30,31} AI-powered smoking cessation apps have also been piloted and rolled out in several settings.³² Beyond personalised behavioural support, AI can help analyse data from wearables and digital health tools to generate behavioural insights, thereby facilitating the design of relevant interventions and the assessment of their impact on health outcomes.

AI can also enhance public communication and engagement efforts by providing more targeted and data-driven strategies and health messages. For example, a range of machine learning techniques can help classify the sentiment (i.e., positive, negative, neutral) of social media posts, providing insights into perceptions of public health messaging.³³ In addition, generative AI has the potential to help synthesise and communicate public health guidance,³⁴ as well as facilitating multilingual communication, particularly with models adapted to specific contexts or languages.

As these examples demonstrate, AI has the potential to support public health systems and practice. However, there are many challenges in deploying these tools and limited evidence in real-world settings on their impacts.

3.3. Barriers to realising AI's potential in public health

We now turn to some of the structural barriers that limit our ability to adopt and influence AI.

- **System fragmentation:** Public health requires multidisciplinary, multi-level approaches spanning national agencies, healthcare delivery systems, local authorities, academia, and civil society.³⁵ Unlike many healthcare systems, which function as a (relatively) coherent system, the public health system often lacks clearly understood pathways to deliver its functions, such as unified procurement or systems to combine data from the many sources on which public health depends. Often described as a strength, its diversity can become a liability when coordinated action is required.
- **Data access and ownership:** Much of the data that would be needed to train, validate and use AI tools for public health purposes (e.g. mobility, wearables, or social media content) lies in the hands of private corporations. Without transparent data-sharing frameworks that create and sustain trust, the public will remain on the margins. Even within the public sector, the type of data that could inform action at the population level is spread across many organisations, such as social care, housing or environmental agencies, which limits the ability to create a cohesive data ecosystem.
- **Acting on what we know:** Critically, for AI insights to deliver meaningful public benefit, they must be actionable. While there is

already a substantial body of evidence on the factors that influence health, such as education or the work environment,³⁶ the challenge often lies not in a lack of knowledge, but in mobilising the necessary resources, political will, and system-wide coordination to act on it. With limited capacity in public health systems, there is a genuine risk that AI will generate vast amounts of data that cannot be used effectively.

- **Hype versus reality:** The discourse surrounding AI in health is often dominated by hyperbolic rhetoric that emphasises the benefits of technological innovation while disregarding the practical challenges of implementation. Too frequently, the focus centres on the capabilities of AI tools rather than the health problems they are intended to address and the realities of the populations they are trying to serve. Before pursuing frontier innovations, public health systems must critically assess whether they have fully leveraged existing statistical methods and computational techniques, and whether a technological or data-driven solution is even needed. The key question remains: Are we addressing the most pressing health challenges, or are we simply chasing technical novelty?
- **Workforce readiness:** AI and digital literacy are uneven across the public health workforce. Without foundational knowledge about the technical, regulatory and ethical dimensions of AI, professionals can be excluded from critical conversations around model design, evaluation, and procurement. This exclusion can lead to over-reliance on external vendors, with resulting deskilling to the public health workforce, and the adoption of AI solutions that may be unsustainable or, at best, offer limited value to the population.
- **Technological limitations:** Despite its potential, AI applications remain constrained by fundamental technical challenges that impact reliability and applicability in health. Issues such as bias, stemming from unrepresentative or incomplete training data, can lead to inequitable outcomes and reinforce existing health disparities.³⁷ Additionally, AI systems, particularly generative models, can generate inaccurate or misleading outputs, often referred to as “hallucinations”.³⁸ These limitations raise serious concerns about the trustworthiness of AI-generated insights, especially in high-stakes public health contexts. This risk may increase, as inputs to large learning models is increasingly AI-generated rather than from human sources. There is already evidence that human language is changing as a consequence of incorporating material generated by AI.³⁹ Without rigorous validation, transparency, and ongoing oversight, there is a concern that AI tools may introduce new risks and exacerbate existing disparities rather than reducing them.

4. Discussion

4.1. What we can do now: practical steps forward

Despite these challenges, there is an opportunity to create the conditions for meaningful engagement of the public health workforce with this evolving technology. A proactive, pragmatic approach should include.

1. **AI guidance, literacy and training:** AI and data science principles should be embedded in public health education. There is no need to create a workforce full of AI experts; however, public health professionals should be empowered to ask critical questions and participate in design, procurement, and governance discussions. Additionally, it is essential to ensure that the workforce can critically appraise and evaluate AI research and applications.⁴⁰ National and regional guidance on how organisations and public health teams can effectively utilise AI, navigate regulatory frameworks, and consider feasibility is needed to ensure a unified public health response.
2. **Multidisciplinary teams:** To ensure AI serves public health effectively, rather than reacting to its unintended consequences, there is a critical need to recruit and partner with experts across an even wider

range of disciplines than usual, including software development, ethics, and behavioural science. Cognitive psychology insights will be especially important as the processes used by human brains and AI are very different.⁴¹ Like any complex public health challenge, leveraging AI requires teams that can collectively navigate technical design, ethical implications, user behaviour, and system integration. AI solutions should be shaped by both the technology and subject matter experts.

3. **Data infrastructure:** Sustainable AI solutions require a well-structured ecosystem encompassing data, computing, and operational capacity. This environment is essential to promote responsible use of AI across its lifecycle. At its core is the need for high-quality, representative data that can be used securely and at scale, for example, through cloud-based, federated environments that protect privacy.⁴² As the healthcare system invests in such infrastructure for clinical data, the organisations involved in delivering public health functions must also consider how to build their foundations before rushing to adopt off-the-shelf AI tools, or before committing the significant resource needed to build bespoke products. The public health community, with its concern about the environment, must also engage with the energy and resources required to operate AI systems.⁴³
4. **Engage communities:** AI design and deployment need to target problems that are grounded in real-life experiences and driven by the needs of the community. Participatory methods that allow the communities to guide AI development and regulation are urgently needed. AI-focused hackathons, innovation labs, or agile efforts in collaboration with national or regional bodies, local authorities, communities, and academic centres can also help shape the public health AI agenda.

Rather than viewing AI as a threat or a panacea, public health professionals must foster a culture that harnesses it as a tool, one that will only be as effective and equitable as the systems that wield it. Without addressing digital exclusion and investing in the building blocks of health, such as social care and housing, the current approach to AI and digital innovation risks missing opportunities to improve population health and may deepen existing inequities.¹⁴ Conversations about AI should begin with a clear understanding of the problem it seeks to solve and an assessment of its added value compared to existing interventions, just as we already do when evaluating other technologies or policies.

4.2. Conclusion

AI is poised to transform public health systems, whether public health stakeholders are at the table or not. If public health professionals do not engage now, ethically, critically, and creatively, we risk becoming spectators to a transformation that directly affects the populations we serve. This is not unprecedented; we have seen the consequences of delayed engagement in public health responses to challenges like tobacco and social media.

Yet we are not starting from zero. The public health community is already accustomed to working across systems, evaluating complex interventions, addressing social injustices, engaging with communities, and navigating uncertainty. These are precisely the lenses needed to guide AI on a path that promotes public good.

This is not a call for digital modernisation or to embark on technology-driven public health. It is a call to ensure that the future of health is not defined solely by those with the most data, the most money, or the most influence, but by those who centre equity, prevention, and the well-being of populations.

Let us not repeat the mistakes of the past. The time to act is now.

Author statements

Ethical approval

This is a commentary and no ethical approval was needed for this work.

Funding

This is a commentary and no funding was needed for this work.

Competing interests

PdRP is employed by the NHS in England. RP is part funded by a University of Oxford Clarendon-Reuben Scholarship and is Chair of the NICE Quality Standards Committee. MM's work on AI is part of the work programme of the European Observatory on Health Systems and Policies, a partnership hosted by WHO. JS is employed by the NHS in England and by the Artificial Intelligence Centre for Value-Based Healthcare in London.

References

- Mistry P. *More than Just Hype: How Emerging AI Use Is Assisting Health and Social Care*. The Kings Fund; 2025.
- Stanford University. *Artificial Intelligence Index Report 2025*. Human-centered AI; 2025. <https://hai.stanford.edu/ai-index/2025-ai-index-report>.
- Esteve A, Chou K, Yeung S, et al. Deep learning-enabled medical computer vision. *npj Digit Med*. 2021;4(1). <https://doi.org/10.1038/s41746-020-00376-2>.
- Buse K, Lim J, Tan C, Kamarulzaman A. Precision public health must get political. *BMJ*. 2025;390, r1471. <https://doi.org/10.1136/bmj.r1471>.
- World Health Organization. Global strategy on digital health 2020-2025. <http://www.who.int/publications/i/item/9789240020924>; 2021. Accessed October 5, 2025.
- World Health Organisation. Global initiative on digital health. Published online <https://www.who.int/initiatives/gidh>; 2024.
- OECD. Artificial intelligence in science: challenges, opportunities and the future of research. Published online <https://doi.org/10.1787/a8d820bd-en>; 2023.
- Global Digital Health Partnership (GDHP). A global commitment to interoperability: the global Digital Health Partnership (GDHP) Interoperability working group report. <https://gdhp.health/wp-content/uploads/2022/03/GDHP-Interop-Final.pdf>; 2022.
- Beaulieu-Jones BK, Yuan W, Brat GA, et al. Machine learning for patient risk stratification: standing on, or looking over, the shoulders of clinicians? *npj Digit Med*. 2021;4(1). <https://doi.org/10.1038/s41746-021-00426-3>.
- Clusmann J, Kolbinger FR, Muti HS, et al. The future landscape of large language models in medicine. *Communications Medicine*. 2023;3(1). <https://doi.org/10.1038/s43856-023-00370-1>.
- Karunanayake N. Next-generation agentic AI for transforming healthcare. *Informatics and Health*. 2025;2(2):73–83. <https://doi.org/10.1016/j.infoh.2025.03.001>.
- NHS England. AI-Enabled ambient scribing products in health and care settings. <https://www.england.nhs.uk/long-read/ai-enabled-ambient-scribing-products-in-health-and-care-settings/>.
- NHS England. NHS transformation directorate. Using an AI chatbot to streamline mental health referrals. Accessed May 2025. Blueprinting. Published online <https://transform.england.nhs.uk/key-tools-and-info/digital-playbooks/workforce-digital-playbook/using-an-ai-chatbot-to-streamline-mental-health-referrals/>; 2025.
- Morley J, Barry E, Hiam L. Can a digital NHS be equitable? *BMJ*. 2025;389, r1317. <https://doi.org/10.1136/bmj.r1317>.
- Hasanzadeh F, Josephson CB, Waters G, Adedinsowo D, Azizi Z, White JA. Bias recognition and mitigation strategies in artificial intelligence healthcare applications. *npj Digit Med*. 2025;8(1). <https://doi.org/10.1038/s41746-025-01503-7>.
- Khalifa M, Albadowy M. AI in diagnostic imaging: revolutionising accuracy and efficiency. *Computer Methods and Programs in Biomedicine Update*. 2024;5(100146). <https://doi.org/10.1016/j.cmpbup.2024.100146>.
- Tierney AA, Gayre G, Hoberman B, et al. Ambient artificial intelligence scribes: learnings after 1 year and over 2.5 million uses. *NEJM Catalyst*. 2025;6(5). <https://doi.org/10.1056/cat.25.0040>.
- Panteli D, Adib K, Buttigieg S, et al. Artificial intelligence in public health: promises, challenges, and an agenda for policy makers and public health institutions. *Lancet Public Health*. 2025;10(5):e428–e432. [https://doi.org/10.1016/s2468-2667\(25\)00036-2](https://doi.org/10.1016/s2468-2667(25)00036-2).
- Fisher S, Rosella LC. Priorities for successful use of artificial intelligence by public health organizations: a literature review. *BMC Public Health*. 2022;22(1). <https://doi.org/10.1186/s12889-022-14422-z>.
- London Secure Data Environment. Discover-NOW. London Secure Data Environment. The Discover Dataset; 2025. Published online <https://discover-now.co.uk/the-data/>.
- Olawade DB, Wada OZ, Ige AO, Egbewole BI, Olojo A, Oladapo BI. Artificial intelligence in environmental monitoring: advancements, challenges, and future directions. *Hygiene and Environmental Health Advances*. 2024;12:100–114. <https://doi.org/10.1016/j.heha.2024.100114>.
- Shajari S, Kuruvinashetti K, Komeili A, Sundararaj U. The emergence of AI-Based wearable sensors for digital health technology: a review. *Sensors*. 23(23):9498. doi: <https://doi.org/10.3390/s23239498>.
- Matthay EC, Neill DB, Titus AR, et al. Integrating artificial intelligence into causal research in epidemiology. *Current Epidemiology Reports*. 2025;12(1). <https://doi.org/10.1007/s40471-025-00359-5>.
- Yang D, Tang Y, Chan VKY, et al. Population-wide depression incidence forecasting comparing autoregressive integrated moving average and vector autoregressive integrated moving average to temporal fusion transformers: longitudinal observational study. e67156. *J Med Internet Res*. 2025;27. <https://doi.org/10.2196/67156>.
- Holm NN, Le TM, Frølich A, et al. amVAE: age-aware multimorbidity clustering using variational autoencoders. 2025;186:109632. *Comput Biol Med*. 2025;186 (109632). <https://doi.org/10.1016/j.combiomed.2024.109632>.
- Krieger M, J Y, E D, et al. An overdose forecasting dashboard for local harm-reduction response. *Health Promot Pract*. 2025. <https://doi.org/10.1177/15248399251335620>. Published online May 5.
- Kraemer MUG, Tsui JLH, Chang SY, et al. Artificial intelligence for modelling infectious disease epidemics. *Nature*. 2025;638(8051):623–635. <https://doi.org/10.1038/s41586-024-08564-w>.
- Laurence T, Harris J, Loman L, et al. Review GIDE – restaurant review gastrointestinal illness detection and extraction with large language models. *arXiv Preprint posted online*. 2025. <https://doi.org/10.48550/arXiv.2503.09743>.
- Cao Q, Jiang R, Yang C, Fan Z, Song X, Shibasaki R. MepoGNN: metapopulation epidemic forecasting with graph neural networks. In: *Lecture Notes in Computer Science* 13718.
- Germani F, Spitale G, Biller-Andorno N. S.A.R.A.H and the decline of trust in health information: a case study. *Health Pol Technol*. 2025;14(3), 101032. <https://doi.org/10.1016/j.hlpt.2025.101032>.
- World Health Organization. S.A.R.A.H, a smart AI resource assistant for health. <https://www.who.int/campaigns/s-a-r-a-h>; 2025.
- Councill Wallsall. Quit smoking with AI coach 'Bella' to quit for good. <https://go.wa.lsal.gov.uk/newsroom/quit-smoking-ai-coach-bella-quit-good>; 2025. Accessed October 8, 2025.
- Valarmathi B, Gupta NS, Karthick V, Chellatamilan T, Santhi K, Chalicheemala D. Sentiment analysis of Covid-19 Twitter data using deep learning Algorithm. *Procedia Comput Sci*. 2024;235:3397–3407. <https://doi.org/10.1016/j.procs.2024.04.320>.
- Lim S, Schmalzle R. Artificial intelligence for health message generation: an empirical study using a large language model (LLM) and prompt engineering. *Frontiers in Communication*. 2023;8. <https://doi.org/10.3389/fcomm.2023.1129082>.
- UK Health Security Agency. *How We Are Pioneering Artificial Intelligence Applications in Public Health* UK Health Security Agency. UKHSA; 2025. <https://ukhsa.blog.gov.uk/2025/03/14/how-we-are-pioneering-artificial-intelligence-applications-in-public-health/>.
- Dahlgren G, Whitehead M. The Dahlgren-Whitehead model of health determinants: 30 years on and still chasing rainbows. *Public health*. 2021;119:20–24. <https://doi.org/10.1016/j.puhe.2021.08.009>.
- Mittermaier M, Raza MM, Kvedar JC. Bias in AI-based models for medical applications: challenges and mitigation strategies. *npj Digit Med*. 2023;6(1). <https://doi.org/10.1038/s41746-023-00858-z>.
- Sun Y, Sheng D, Zhou Z, Wu Y. AI hallucination: towards a comprehensive classification of distorted information in artificial intelligence-generated content. *Humanit Soc Sci Commun*. 2024;11(1). <https://doi.org/10.1057/s41599-024-03811-x>.
- Yakura H, Lopez-Lopez E, Brinkmann L, Serna I, Gupta P, Rahwan I. Empirical evidence of Large language Model's influence on human spoken communication. *arXiv preprint arXiv:2409.1754*. 2024. <https://doi.org/10.48550/arXiv.2409.01754>. Preprint posted online 2024.
- Dijkstra P, Greenhalgh T, Magdi Mekki Y, Morley J. How to read a paper involving artificial intelligence (AI). *BMJ Medicine*. 2025;4(1), e001394. <https://doi.org/10.1136/bmjmed-2025-001394>.
- Rosenbacke R, Melhus Å, McKee M, Stuckler D. AI and XAI second opinion: the danger of false confirmation in human-AI collaboration. *J Med Ethics*. 2025;51(6): 396–399. <https://doi.org/10.1136/jme-2024-110074>.
- One London. *A framework for the safe, efficient and effective implementation, use and maintenance of AI in health and care in London*. Chapter 4; 2025. <https://www.onelondon.online/wp-content/uploads/2025/03/A-Framework-for-the-safe-efficient-and-effective-implementation-use-and-maintenance-of-AI-in-health-and-care-in-London.pdf>.
- Hamilton E. Inclusionary zoning and housing market outcomes. *Citiescape*. 2021;23 (1):161–194.